

Calendar Line # 6
Case Name Jennifer Voges et al vs Huiqiong Deng et al
Case No. 25CV483365
Demurrer Before the court is defendants Stanford Health Care and Huiqiong Deng, M.D.’s demurrer to the complaint. Pursuant to California Rule of Court 3.1308, the court issues its tentative ruling as follows. I. <u>BACKGROUND</u> Plaintiff Jennifer Voges (“Voges”) was a long-term prescribed benzodiazepine user and was in active benzodiazepine withdrawal when she sought and received care from defendant El Camino Health dba El Camino Hospital (“ECH”). (Complaint, ¶8). Despite plaintiff Voges’s benzodiazepine dependence and active withdrawal, defendant ECH staff administered opiates, including morphine and Dilaudid (hydromorphone), creating a high risk combination known to increase respiratory and neurologic dangers. (Complaint, ¶9). As a result of the contradicted opioids, plaintiff Voges suffered a drug-induced crisis requiring Narcan to reverse opiate toxicity and she was then given anti-psychotic medication that is contraindicated in benzodiazepine withdrawal, further destabilizing her condition. (Complaint, ¶10). Plaintiff Voges required Narcan after opiate administration and was given antipsychotic medication that worsened her condition. (<i>Id.</i>). Defendant ECH failed to obtain informed consent for, or disclose, the Narcan administration during or after the event. (Complaint, ¶11). Defendant ECH never informed plaintiff Voges or her family that Narcan had been administered, did not obtain consent, and did not disclose the intervention afterward. (<i>Id.</i>). Plaintiff Voges and her family later discovered this only upon review of medical records. (<i>Id.</i>). Defendant ECH staff refused to provide plaintiff Voges’s baseline, prescribed, benzodiazepine and refused her progesterone, despite documented prescription and visible withdrawal symptoms, accelerating her deterioration. (Complaint, ¶12). Plaintiff Voges received care at defendant ECH between December 31, 2024 and January 8, 2025. (Complaint, ¶2). Rather than stabilizing plaintiff Voges, defendant ECH transferred her to defendant Aspen Skilled Healthcare, Inc. dba Woodlands Healthcare Center (“Woodlands”), a skilled nursing facility, for physical therapy even though she was medically unstable and required medical, not rehabilitative, care beyond the scope of a skilled nursing facility. (Complaint, ¶12). Defendant Woodlands lacked qualified personnel to properly manage benzodiazepine withdrawal and opiate complications. (<i>Id.</i>). Plaintiff Voges’s husband ultimately removed plaintiff Voges and transported her to defendant Stanford Health Care dba Stanford Hospital (“Stanford”) where she was immediately admitted. (Complaint, ¶15). At Stanford, under the care of defendant Huiqiong Deng, M.D. (“Dr. Deng”), plaintiff Voges was subjected to the administration of Ketamine against her explicit refusals; repeated invasive testing represented as “mandatory” despite objections; and undisclosed administration of medications, including ciprofloxacin, contraindicated in benzodiazepine withdrawal. (Complaint, ¶16). Stanford personnel, including Dr. Deng, engaged in coercive and intimidating conduct, including threats of psychiatric transfer for refusing medications, forced psychiatric evaluations despite objections, and threats of a feeding tube when plaintiff could not eat due to benzodiazepine related gastrointestinal symptoms. (Complaint, ¶17). Defendants Stanford and/or Dr. Deng failed to disclose, evaluate, or treat a documented enlarged thyroid observed on imaging, did not provide inpatient endocrine evaluation, and later canceled scheduled endocrinology follow-up. (<i>Id.</i>). A subsequent outside endocrinology evaluation in December 2025 noted a significant enlarged, tender, misshapen thyroid with concern for malignancy. (<i>Id.</i>)

On December 30, 2025, plaintiffs Voges and Darren Voges filed a complaint against defendants Stanford and Dr. Deng, among others, asserting causes of action for:

- (1) Professional Negligence
- (2) Medical Battery
- (3) Negligent Hiring, Supervision, and Retention
- (4) Negligence Per Se
- (5) Loss of Consortium

On June 30, 2026, defendants Stanford and Dr. Deng filed the motion now before the court, a demurrer to the second and fourth causes of action of plaintiffs' complaint.

II. LEGAL STANDARD

Pursuant to Code of Civil Procedure section 430.10, a party may demur to a complaint on the grounds that it “does not state facts sufficient to constitute a cause of action.” (Code Civ. Proc., § 430.10, subd. (e)). A demurrer tests whether the complaint states a cause of action. (*Hahn v. Mirda* (2007) 147 Cal.App.4th 740, 747 (*Hahn*)). When considering demurrers, courts accept all well pleaded facts as true. (*Fox v. JAMDAT Mobile, Inc.* (2010) 185 Cal.App.4th 1068, 1078). In ruling on a demurrer, the Court treats it “as admitting all material facts properly pleaded, but not contentions, deductions or conclusions of fact or law.” (*Piccinini v. Cal. Emergency Management Agency* (2014) 226 Cal.App.4th 685, 688, citing *Blank v. Kirwan* (1985) 39 Cal.3d 311, 318 (*Blank*)).

III. ANALYSIS

A. MEDICAL BATTERY

Our high court has made it clear that battery and lack of informed consent are separate causes of action. A claim based on lack of informed consent—which sounds in negligence—arises when the doctor performs a procedure without first adequately disclosing the risks and alternatives. In contrast, a battery is an intentional tort that occurs when a doctor performs a procedure without obtaining any consent. (*Saxena v. Goffney* (2008) 159 Cal.App.4th 316, 324).

Plaintiff Voges's second cause of action is labeled as one for medical battery but then alleges, in relevant part, “Defendants, including Deng, performed medical interventions without obtaining [plaintiff Voges's] informed consent or by failing to disclose material information, risks, alternatives, or by misrepresenting necessity, including . . . Ketamine at Stanford despite express refusal. . . . Stanford administered Ketamine despite explicit refusals and concealed medication identities including ciprofloxacin.” (Complaint, ¶26).

In demurring, defendants Stanford and Dr. Deng highlight plaintiff Voges's allegation that she presented with “cognitive impairment” and active benzodiazepine withdrawal (see Complaint, ¶¶13 and 15) but never alleges she regained cognitive abilities. Defendants contend the allegations of the complaint establish plaintiff was “disputing the professional medical judgments of the healthcare providers,” but plaintiff Voges does not allege defendants Stanford or Dr. Deng intentionally violated the consent which was provided.

Although the court may draw reasonable inferences from the explicit allegations of a complaint, the court must also accept, as true, the allegations of the pleading. Defendants Stanford and Dr. Deng essentially ask the court to infer from the allegation that plaintiff Voges presented with “cognitive impairment” (at Woodlands, prior to her admission to Stanford) that she never regained any cognitive abilities. However, the court will not make such an inference here as plaintiff Voges alleges that while at Stanford, she “explicitly refus[ed]” the administration of Ketamine. Such an allegation is an express allegation that defendants Stanford and Dr. Deng intentionally violated the consent plaintiff Voges provided.

Moreover, as plaintiff Voges points out in her opposition, the complaint further alleges she was subjected to “undisclosed administration of medications, including ciprofloxacin.” (Complaint, ¶16). The reasonable inference to be drawn from the allegation that such administration was “undisclosed,” is that the drugs were administered without plaintiff Voges’s knowledge and, consequently, without plaintiff Voges’s consent. The allegation of an “undisclosed administration of medications” is sufficient to support plaintiff Voges’s second cause of action for medical battery.

Accordingly, defendants Stanford and Dr. Deng’s demurrer to the second cause of action of plaintiff Voges’s complaint on the ground that the pleading does not state facts sufficient to constitute a cause of action [Code Civ. Proc., §430.10, subd. (e)] for medical battery is **OVERRULED**.

B. NEGLIGENCE PER SE

Plaintiff Voges’s fourth cause of action, entitled “Negligence Per Se,” alleges, in relevant part, that defendants “violated patient-protection duties requiring informed consent for non-emergent interventions and post-emergency disclosure/ documentation of medications administered ... and disclosure of material findings.” (Complaint, ¶34).

“Section 669 of the Evidence Code sets forth the doctrine commonly called negligence per se. It provides that negligence of a person is presumed if he violated a statute or regulation of a public entity, if the injury resulted from an occurrence that the regulation was designed to prevent, and if the person injured was within the class for whose protection the regulation was adopted. This presumption may be rebutted by proof that the violator did what might reasonably be expected of a person of ordinary prudence, acting under similar circumstances, who desired to comply with the law.” (*Klein v. BIA Hotel Corp.* (1996) 41 Cal.App.4th 1133, 1140). Negligence per se is not a cause of action. “The negligence per se doctrine actually relates to the burden of proof.” (*Cade v. Mid-City Hospital Corp.* (1975) 45 Cal.App.3d 589, 596). If the elements are met, the doctrine merely creates an evidentiary presumption in support of a claim for negligence.

“ ‘[T]he doctrine of negligence per se is not a separate cause of action, but creates an evidentiary presumption that affects the standard of care in a cause of action for negligence.’ [Citation.] [¶] The doctrine of negligence per se does not provide a private right of action for violation of a statute. [Citation.]” (*Johnson v. Honeywell Intern. Inc.* (2009) 179 Cal.App.4th 549, 555–56).

Of course, plaintiff Voges will be allowed to pursue/ apply the doctrine of negligence per se at time of trial, but since negligence per se is not a cause of action and plaintiff Voges has already stated a cause of action for professional negligence, defendants Stanford and Dr. Deng’s demurrer to the fourth cause of action of plaintiff Voges’s complaint on the ground that the pleading does not state facts sufficient to constitute a cause of action [Code Civ. Proc., §430.10, subd. (e)] for negligence per se is **SUSTAINED WITHOUT LEAVE TO AMEND**.

IV. CONCLUSION

Based on the foregoing, the Court Orders as follows: (1) Defendants Stanford and Dr. Deng’s demurrer to the second cause of action of plaintiff Voges’s complaint for medical battery is **OVERRULED**; and (2) Defendants Stanford and Dr. Deng’s demurrer to the fourth cause of action of plaintiff Voges’s complaint for negligence per se is **SUSTAINED WITHOUT LEAVE TO AMEND**. The Court will prepare the formal Order.

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Calendar Lines # 7-8
Case Name James Souza III vs Victoria Garcia
Case No. 26CV493764
Motion to Quash (Line # 7) and Demurrer (Line #8) Before the court is the defendant Motion to Quash Service and Summons (Line #7) and Demurrer (Line #8). Pursuant to California Rule of Court 3.1308, the court issues its tentative ruling as follows. <u>MOTION TO QUASH (LINE # 7)</u> I. <u>BACKGROUND</u> On June 30, 2026, Defendant Victoria Garcia (“Garcia”) filed a motion to quash service of summons. The motion was accompanied by a proof of service indicating mail service to Plaintiff James Souza III (“Souza”) on July 6, 2026. The motion is unopposed. Per Code of Civil Procedure section 1005(b) opposition papers were due on July 22, 2026. A failure to oppose a motion may be deemed a consent to the granting of the motion. California Rule of Court Rule 8.54c. A failure to oppose a motion may be deemed a consent to the granting of the motion. (California Rule of Court Rule 8.54(c)). Failure to oppose a motion leads to the presumption that the defendant has no meritorious arguments. (<i>Laguna Auto Body v. Farmers Ins. Exchange</i> (1991) 231 Cal.App.3d 481, 489). The Court has carefully reviewed the Defendant’s moving papers, including, the notice of motion (totaling 2 pages); memorandum of points and authorities (totaling 5 pages); Declaration of Robert T. Meastas and attached Exhibits A-B (totaling 35 pages); proof of services; and the pleadings. II. <u>LEGAL STANDARD</u> Pursuant to Code of Civil Procedure section 418.10, a defendant may file a motion to quash service of a summons “on the ground of lack of jurisdiction of the court over him or her.” (Code Civ. Proc., § 418.10, subd. (a)(1)). “When a defendant moves to quash service of process on jurisdictional grounds, the plaintiff has the initial burden of demonstrating facts justifying the exercise of jurisdiction.” (<i>Vons Companies. v. Seabest Foods</i> (1996) 14 Cal.4th 434, 449; accord <i>Snowney v. Harrah’s Entertainment</i> (2005) 35 Cal.4th 1054, 1062; <i>Swenberg v. Dmarcian, Inc.</i> (2021) 68 Cal.App.5th 280, 291; <i>Lebel v. Mai</i> (2012) 210 Cal.App.4th 1154, 1163; <i>Elkman v. National States Ins. Co.</i> (2009) 173 Cal.App.4th 1305, 1313; <i>Summers v. McClanahan</i> (2006) 140 Cal.App.4th 403, 413). “If the plaintiff meets this initial burden, then the defendant has the burden of demonstrating that the exercise of jurisdiction would be unreasonable.” (<i>Pavlovich v. Super. Ct.</i> (2002) 29 Cal.4th 262, 273; accord <i>Snowney, supra</i>, 35 Cal. 4th at p. 1062; <i>Vons Companies, supra</i>, 14 Cal.4th at p. 449; <i>Swenberg, supra</i>, 68 Cal.App.5th at p. 291). Code of Civil Procedure section 410.10, the court “may exercise jurisdiction on any basis not inconsistent with the Constitution of this state or of the United States.” A court may exercise jurisdiction over a defendant if the plaintiff presents sufficient jurisdictional evidence to establish that “the defendant has such minimum contacts with California that the assertion of jurisdiction does not violate traditional notions of fair play and substantial justice.” (<i>Bruce v. Shabaab</i> (2020) 54 Cal.App.5th 578, 589). There are two different kinds of personal jurisdiction: general jurisdiction and specific jurisdiction. General jurisdiction applies when a defendant is domiciled in the forum or is otherwise “essentially at home.” (<i>Ford Motor Co. v. Montana Eighth Judicial Dist. Ct.</i> (2021) 592 U.S. 351, 358; see also <i>Daimler AG v. Bauman</i> (2014) 571 U.S. 117, 137). Specific jurisdiction, in contrast, examines the relationship between the defendant’s contacts with the forum state and the causes of action asserted in the litigation; it generally requires that there be some evidence of “purposeful availment” or other connection between the defendant and the forum and that the litigation arises out of the defendant’s forum-related contacts. (<i>Ford Motor Co., supra</i>, 592 U.S. at pp. 359-360). Jurisdictional facts must